

DAY 1 STUDY GUIDE: EARLY PREGNANCY COMPLICATIONS

Bleeding Disorders, Abortions, & Hyperemesis Gravidarum

Session 1 of 4 | Wednesday, October 14, 2026 | 11:00 AM - 1:20 PM

01. WHAT YOU WILL LEARN TODAY

1. Compare early vs. late pregnancy bleeding and their underlying clinical causes.
2. Differentiate classic types of spontaneous abortion and map immediate nursing priorities.
3. Recognize and prioritize care for ectopic pregnancy, gestational trophoblastic disease (GTD), and cervical insufficiency.
4. Contrast placenta previa and placental abruption, applying time-sensitive clinical nursing actions.
5. Relate Disseminated Intravascular Coagulation (DIC) pathophysiology and labs to severe obstetric hemorrhage.
6. Safe administration and assessment guidelines for critical pregnancy loss and abortion-related medications.
7. Prioritize clinical care, step-by-step rehydration, and lifestyle modifications for hyperemesis gravidarum.

02. BLEEDING IN PREGNANCY: CLINICAL TIMING

Bleeding at any point in pregnancy is a critical, potentially life-threatening event. The leading cause of maternal mortality worldwide is **obstetric hemorrhage**. Clinical management always relies on a rapid sequence of: **Early Recognition > Astute Nursing Assessment > Immediate Resuscitation & Stabilization**.

TIMING WINDOW	COMMON CAUSES	CLINICAL HALLMARK & CHARACTERISTICS
EARLY BLEEDING (First Half / Trimester 1 & 2 Before 20 Weeks)	• Spontaneous Abortion • Ectopic Pregnancy • Gestational Trophoblastic Disease (GTD) • Cervical Insufficiency	Vaginal bleeding, spotting, or cramping before 20 weeks of gestation. Typically localized pelvic discomfort without uterine rigidity.
LATE BLEEDING (Second Half / Trimester 3 After 20 Weeks)	• Placenta Previa • Placental Abruption • Placenta Accreta Spectrum	Vaginal bleeding accompanied by alterations in uterine tone (softness vs. board-like rigidity) and high risk of immediate fetal compromise.

03. SPONTANEOUS ABORTION (MISCARRIAGE)

Spontaneous abortion (miscarriage) is defined as the loss of an early pregnancy from natural causes before **week 20 of gestation**. It is the most common early pregnancy complication, occurring in up to 31% of first-trimester pregnancies. Loss after week 20 is classified as a stillbirth.

ABORTION TYPE	VAGINAL BLEEDING	CERVIX STATUS	CLINICAL MEANING & NURSING INTERVENTIONS
Threatened	Present; mild spotting	Closed	Pregnancy may continue. Bed rest, avoid stress, and monitor hCG/ultrasound closely.
Inevitable	Present; moderate-heavy	Dilated	Pregnancy loss cannot be stopped. Prepare client for impending tissue passage.
Incomplete	Heavy; severe cramping	Dilated	Some products of conception (POC) remain. High risk of hemorrhage. Prepare for D&C/suction curettage.
Complete	Decreased; mild cramping	Closed	All products of conception have passed naturally. Uterus is empty. Monitor for infection.

ABORTION TYPE	VAGINAL BLEEDING	CERVIX STATUS	CLINICAL MEANING & NURSING INTERVENTIONS
Missed	Absent or dark brown spotting	Closed	Fetus has died but is retained in utero. Risk of DIC if prolonged. Prepare for medical/surgical evacuation.
Septic	Present; foul-smelling	Dilated	Infection of retained tissue. High fever, chills, uterine tenderness. Emergency broad-spectrum antibiotics and evacuation.

CRITICAL NURSING ACTIONS & ASSESSMENTS FOR EARLY LOSS

- **Vaginal Bleeding:** Assess color (bright red is highly significant) and amount by counting and weighing peripads (saturating 1 peripad hourly is significant). Ask about passage of clots/tissue and **instruct client to save passed tissue**.
- **Hemodynamic Monitoring:** Monitor vital signs, pain, and cramping. Assess for early signs of hypovolemic shock (tachycardia, tachypnea, hypotension, anxiety).
- **Medical/Surgical Prep:** Prepare client for suction curettage / dilation & curettage (D&C) or medical evacuation (e.g., misoprostol).
- **Isoimmunization Prevention:** Administer **Rho(D) immune globulin (RhoGAM) within 72 hours** if client is Rh-negative and non-sensitized.
- **Grief Support:** Reassure the client that spontaneous abortions usually result from chromosomal abnormalities and are **not caused by their actions**. Validate acute grief, allow verbalization, and refer to community support groups.

04. ECTOPIC PREGNANCY

An **ectopic pregnancy** occurs when a fertilized ovum implants outside the main uterine cavity. Because **96% of ectopic pregnancies implant within the fallopian tubes**, they carry a catastrophic risk of tubal rupture, massive intra-abdominal hemorrhage, shock, future infertility, or death.

- **High-Risk Etiology & Risk Factors:** Previous ectopic pregnancy (triples risk), history of Pelvic Inflammatory Disease (PID) or sexually transmitted infections (STIs causing tubal scarring), endometriosis, in-utero exposure to diethylstilbestrol (DES), cigarette smoking, prior tubal surgeries, or use of assisted reproductive technology (IVF).
- **Diagnostic Triad:** Low beta-hCG levels (drawn serially), transvaginal ultrasound showing an extrauterine gestational sac, and a palpable adnexal mass/tenderness.
- **Medical Management (Unruptured / Stable):** Administer **methotrexate** (folic acid antagonist that inhibits rapid cell division). Criteria: hemodynamically stable, mass <4 cm, no active peritoneal bleeding, low beta-hCG levels. *Nursing:* Counsel on side effects (N/V, stomatitis, gastric upset). Monitor beta-hCG titers on Days 4 and 7; monitor weekly until undetectable.
- **Surgical Management (Ruptured / Unstable):** Emergency laparotomy. Perform a **salpingectomy** (removal of affected fallopian tube) if ruptured to control active hemorrhage. If unruptured and future fertility is highly desired, a **linear salpingostomy** is preferred to preserve the tube.
- **Contraceptive Guidance:** Instruct the client to use reliable contraception for **at least 3 menstrual cycles** to allow the reproductive tract and fallopian tubes to heal fully.

NGN CLINICAL JUDGMENT ALERT: ECTOPIC RUPTURE CUES

CLASSIC PRESENTATION (NGN HALLMARK CUE): Unilateral abdominal pain with vaginal spotting occurring **6 to 8 weeks after a missed menstrual period**. Undergo rapid assessment immediately!

CRITICAL DANGER SIGN (TUBAL RUPTURE): Sudden, sharp, stabbing, unilateral pelvic pain accompanied by **referred shoulder pain** (caused by blood pooling in the peritoneal cavity, irritating the phrenic nerve). Watch for rapid progression to hypovolemic shock (hypotension, tachycardia, cold clammy skin, vertigo/fainting). This is a **surgical emergency!** Act immediately!

05. GESTATIONAL TROPHOBLASTIC DISEASE (GTD / MOLAR PREGNANCY)

Gestational Trophoblastic Disease (GTD) represents a spectrum of benign and malignant neoplastic disorders arising from abnormal hyperproliferation of trophoblastic (placental) cells. The pregnancy is non-viable. These cells produce massive amounts of **human chorionic gonadotropin (hCG)**, which serves as the primary clinical marker for diagnosis, surveillance, and recurrence.

FEATURE	COMPLETE HYDATIDIFORM MOLE (80% OF CASES)	PARTIAL HYDATIDIFORM MOLE
Pathophysiology & Origin	An 'empty' maternal egg is fertilized by 1 or 2 normal sperm. Contains 46 all-paternal chromosomes . No fetal tissue or amniotic sac develops.	Two sperm fertilize a normal egg, leading to double contribution. Results in a triploid karyotype (69 chromosomes) . May have minimal fetal parts, but non-viable.
Clinical Manifestations	Vaginal bleeding (dark brown 'prune-juice' or bright red), pelvic pressure, uterus larger than expected for gestational dates , severe hyperemesis, and preeclampsia before 20 weeks (classic red flag).	Often presents as an incomplete or missed abortion with irregular vaginal bleeding, milder uterine enlargement, and cramping.
Malignant Potential	Higher risk of transforming into highly invasive choriocarcinoma .	Lower malignant transformation rate, but still requires strict serial surveillance.

SURVEILLANCE & RECURRENCE TEACHING FOR MOLAR PREGNANCY

- **Immediate Evacuation:** Immediate surgical evacuation via **suction D&C** (or hysterectomy if childbearing is complete) to prevent severe hemorrhage and reduce malignant risk.
- **Malignancy Surveillance:** Monitor serial serum beta-hCG levels weekly until they are undetectable, then monthly for 1 year. *Choriocarcinoma Danger Cue:* Shortness of breath (80% of choriocarcinoma cases metastasize to the lungs).
- **CRITICAL CONTRACEPTIVE WARNING:** Instruct the client to utilize a highly reliable contraceptive method to **strictly avoid pregnancy for at least 1 year (6 to 12 months)**. A new pregnancy would cause rising hCG levels, completely masking the detection of choriocarcinoma recurrence.

06. CERVICAL INSUFFICIENCY

Cervical insufficiency is characterized by **painless cervical dilation in the second trimester** without uterine activity or contractions, leading to the rapid expulsion of an immature, non-viable fetus.

- **Risk Factors:** Previous cervical trauma or lacerations (e.g., LEEP procedure, conization, rapid labor), history of prior second-trimester pregnancy loss, in-utero exposure to diethylstilbestrol (DES), or connective tissue disorders (Marfan syndrome, Ehlers-Danlos).
- **Assessment Findings:** Painless dilation, pelvic pressure, pink-tinged vaginal discharge, increase in low backache, and leaking of amniotic fluid.
- **Surgical Cerclage:** Placement of a heavy, purse-string suture to reinforce and close the internal os of the cervix in the second trimester (withdrawn at term for vaginal birth).
- **Nursing Care:** Place client on bed rest and activity restrictions. Monitor closely for signs of preterm labor (cramping, backache, pelvic pressure, or rupture of membranes). Report any changes immediately.

07. LATE PREGNANCY BLEEDING: PREVIA VS. ABRUPTION

Bleeding in the second half of pregnancy (after 20 weeks) is a critical obstetric emergency. The nurse must immediately differentiate **placenta previa** (placenta covering the os) from **placental abruption** (premature placental separation).

CLINICAL FEATURE	PLACENTA PREVIA (MATERNAL ORIGIN)	PLACENTAL ABRUPTION (MATERNAL-FETAL FOCUS)
Onset & Timing	Insidious and gradual; typically after 20 weeks.	Sudden, unexpected, and intense; high-risk emergency.
Bleeding Characteristics	Always visible ; slight to profuse; bright red color.	Can be concealed (behind placenta) or visible ; dark, port-wine color.
Pain & Tenderness	Completely painless ; no abdominal discomfort.	Constant, severe pain ; 'knifelike' abdominal discomfort; extreme uterine tenderness.
Uterine Tone & Palpation	Soft and relaxed ; uterus is non-tender.	Firm, rigid, board-like ; hypertonic resting tone.
Fetal Status & Presentation	Fetal heart rate is usually within normal range; engagement is absent (breech or transverse lie is common).	Fetal distress (Category II/III) or absent fetal heart sounds (fetal death). No presentation relationship.
Primary Risk Factors	Prior placenta previa, prior cesarean birth, multiparity, multiple gestation, increasing maternal age, and maternal smoking/cocaine use.	Hypertension (gestational, preeclampsia, chronic) , abdominal/pelvic trauma, cocaine use, tobacco smoking, and rapid uterine decompression.

CRITICAL NURSING INTERVENTIONS FOR LATE BLEEDING

PLACENTA PREVIA:

- **NEVER PERFORM A VAGINAL EXAMINATION:** Digital vaginal exams can disrupt the placenta, leading to rapid, fatal maternal hemorrhage. Avoid vaginal exams entirely after 20 weeks until previa is ruled out by transvaginal ultrasound.
- **Care Measures:** Establish large-bore IV access, obtain type and crossmatch for at least 2 units of packed RBCs, institute bed rest, continuous electronic fetal monitoring, avoid nipple stimulation (prevents oxytocin release/contractions), and plan for a scheduled cesarean section at term.

PLACENTAL ABRUPTION:

- **EMERGENCY SEQUENCE:** Establish **two large-bore IV lines** (16 or 18G) with normal saline or lactated Ringer's. Obtain urgent type and crossmatch, CBC, and coagulation panel. Place client in **left lateral position** to maximize uterine perfusion. Administer oxygen via nasal cannula. Monitor fundal height (increasing height indicates concealed bleeding). Prepare for immediate emergency cesarean birth if fetal distress is present.

NGN CLINICAL REASONING ALERT: HEMODYNAMIC SHOCK

VITAL SIGNS CAN DECEIVE IN PREGNANCY: Maternal blood volume increases by 50% during pregnancy. A pregnant client can lose up to **30% to 40% of their total blood volume without showing any signs of shock** (such as hypotension or bradycardia). Do not wait for a drop in blood pressure to recognize severe postpartum/anteartum hemorrhage. Monitor for subtle compensatory cues: mild tachycardia, tachypnea, oliguria, anxiety, and cool clammy extremities.

08. DISSEMINATED INTRAVASCULAR COAGULATION (DIC)

Disseminated Intravascular Coagulation (DIC) is a life-threatening, secondary bleeding disorder characterized by an abrupt loss of balance between the clot-forming activity of thrombin and the clot-lysing activity of plasmin. This leads to widespread microvascular clotting (consuming all clotting factors and platelets), followed immediately by **severe, generalized systemic hemorrhage**.

- **Obstetric Triggers:** Placental abruption (most common, due to release of tissue thromboplastin), amniotic fluid embolism (AFE), endotoxin sepsis after abortion, retained dead fetus, hydatidiform mole, HELLP syndrome, and severe posthemorrhagic shock.
- **NGN Diagnostic Lab Panel (Crucial for NCLEX):**
 1. **Platelet Count:** Decreased (thrombocytopenia)

2. **Fibrinogen Levels:** Decreased

3. **Prothrombin Time (PT) & activated Partial Thromboplastin Time (aPTT):** Prolonged / Increased

4. **D-Dimer Test & Fibrin Degradation Products (FDPs):** Strongly Positive (objective evidence of simultaneous clotting & lysis)

• **Emergency Nursing Actions:** Treat the underlying cause immediately (e.g., extract fetus/placenta). Initiate aggressive fluid resuscitation with crystalloids, and administer blood products as ordered (packed red blood cells, fresh-frozen plasma to replace factors, cryoprecipitate to replace fibrinogen, and platelets). Monitor hourly urine output via indwelling Foley catheter (maintain >30 mL/hour). Watch for bleeding gums, oozing from IV sites, petechiae, and epistaxis.

09. TIME-SENSITIVE CLINICAL PHARMACOLOGY

Specific medications are utilized to manage early pregnancy loss, spontaneous/induced abortions, and to prevent maternal isoimmunization. The nurse must master the safe administration and critical assessment parameters of these agents.

MEDICATION	ACTION & CRITICAL INDICATIONS	NURSING IMPLICATIONS & SAFETY ASSESSMENTS
Misoprostol (Cytotec) <i>Prostaglandin Analog</i>	Stimulates uterine contractions to terminate pregnancy or evacuate the uterus after spontaneous abortion to ensure complete passage of products of conception.	- Monitor for gastrointestinal side effects: severe diarrhea, abdominal pain, nausea, vomiting, dyspepsia. - Assess vaginal bleeding: report sudden increases, severe pain, or fever. - Monitor for signs of hypovolemic shock (tachycardia, hypotension, anxiety).
Mifepristone (Korlym) <i>Progesterone Antagonist</i>	Acts as a progesterone antagonist, causing the endometrium to slough; allowed to be followed by misoprostol within 48 hours to stimulate contractions.	- Monitor for headache, severe vomiting, diarrhea, and heavy bleeding. - Anticipate and administer an antiemetic prior to use to reduce nausea/vomiting. - Encourage the use of acetaminophen (not aspirin) to manage cramping discomfort.
Dinoprostone (Prostin E2, PGE2) <i>Prostaglandin E2</i>	Stimulates uterine contractions, causing expulsion of uterine contents in second-trimester fetal death or spontaneous abortion; effaces and dilates cervix.	- Bring the drug to room temperature prior to administration. - Administer suppository high in the vagina using strict sterile technique. - Keep client completely supine for 10 minutes after administration. - Avoid contact with skin (wear gloves; wash immediately if contact occurs).
Rho(D) Immune Globulin (RhoGAM / MICRhoGAM) <i>Passive Immunizing Agent</i>	Suppresses maternal immune response to prevent D-alloimmunization in Rh-negative, non-sensitized clients exposed to Rh-positive blood after abortion, ectopic pregnancy, or trauma.	- Administer intramuscularly (IM) strictly in the deltoid muscle. Dosing: Mini-dose (MICRhoGAM) for abortion/loss up to 12 weeks gestation; full dose (RhoGAM) after 12 weeks gestation. - Check maternal antibody titers (Indirect Coombs) prior to administration (must be negative; ineffective if client is already sensitized).

10. HYPEREMESIS GRAVIDARUM

Hyperemesis gravidarum is a severe, intractable form of nausea and vomiting of pregnancy. It is differentiated from typical 'morning sickness' by its persistence beyond Trimester 1, severe metabolic consequences, and the critical danger of acute maternal starvation. It results in **dehydration, weight loss >5% of prepregnancy body weight, electrolyte imbalances (hypokalemia, hyponatremia), ketosis, ketonuria, and severe nutritional deficiencies.**

PHASE	CLINICAL INTERVENTION	NGN CLINICAL REASONING & NURSING FOCUS
Step 1: Priority	Maintain strict NPO status for the first 24 to 36 hours. Withhold all oral foods, fluids, and oral medications.	PRIORITY CLINICAL REASONING: The priority is to stop all oral intake to allow the gastrointestinal tract and stomach to rest. Forcing fluids/foods by mouth worsens vomiting, accelerating dehydration.
Step 2: Rehydrate	Initiate IV fluid replacement therapy using Normal Saline (to combat hyponatremia) or Lactated Ringer's. Add vitamins (pyridoxine / B6) and electrolytes.	ELECTROLYTE RESTORATION: Corrects fluid volume deficit, prevents hyponatremic encephalopathy, restores potassium, and replaces essential B-complex vitamins used during starvation.
Step 3: Control	Administer prescribed antiemetics rectally or intravenously: promethazine (Phenergan) , ondansetron (Zofran) , or combination pyridoxine/doxylamine (Diclegis) .	PHARMACOLOGIC COMFORT: Promethazine acts on the CTZ; monitor for sedation, hypotension, and urinary retention. Ondansetron blocks serotonin; monitor liver function. Diclegis must be taken daily.
Step 4: Reintroduce	As vomiting subsides, gradually reintroduce oral fluids and foods in very small, frequent amounts .	TOLERANCE CHECK: Monitor intake and output strictly, checking urine for ketones. Transition client to small dry solids (dry crackers/toast) before introducing liquids.
Step 5: Escalate	If intractable vomiting and malnutrition persist after several days of bed rest and IV fluids, initiate Total Parenteral Nutrition (TPN) or PEG-tube feeding.	MALNUTRITION PREVENTION: Prevents severe muscle wasting, maternal liver/kidney dysfunction, and severe fetal growth restriction (FGR).

CLIENT TEACHING FOR NAUSEA & VOMITING (TEACHING GUIDELINES 19.1)

- **Meal Patterns:** Eat small, frequent meals throughout the day (5 to 6 small meals). **Separate fluids from solids** by consuming fluids strictly between meals rather than with them.
- **Dietary Selection:** Consume dry, bland foods that settle the stomach (dry crackers, toast, baked potatoes, soda). Avoid high-fat, greasy, highly spiced, or extremely sweet foods. Use high-protein supplement drinks.
- **Herbal Remedies & Supplements:** Take ginger supplements (250 mg four times daily) or drink peppermint or ginger herbal teas. Take vitamin B6 (pyridoxine) as prescribed.
- **Environmental & Postural Changes:** Avoid noxious stimuli (strong frying odors, bacon, perfumes, paints) that trigger nausea. **Do not lie down or recline for at least 2 hours after eating** to prevent acid reflux. Wear loose-fitting waistbands.
- **Rest & Stress Management:** Schedule daily rest periods to avoid becoming overtired. Learn stress management techniques, as physical fatigue and psychological stress directly trigger hyperemesis flares.

11. NEXT-GEN NCLEX (NGN) SELF-CHECK QUESTIONS

Answer the following six clinical reasoning questions to evaluate your mastery of RNSG 2514 Day 1 content. Check your clinical rationales on the following page.

Q1. A 39-year-old G5P4 multigravida client at 32 weeks' gestation presents to the labor suite with sudden-onset, painless, bright-red vaginal bleeding. Her vital signs are stable, and the uterus is soft, relaxed, and non-tender. Which nursing action is strictly CONTRAINDICATED in the immediate management of this client?

- A. Placing the client in a left lateral side-lying position.
- B. Establishing a large-bore intravenous line (18G) with Lactated Ringer's.
- C. Performing a digital vaginal examination to assess cervical dilation.
- D. Obtaining a type and crossmatch for 2 units of packed red blood cells.

Q2. A client at 7 weeks' gestation presents to the emergency department reporting mild vaginal spotting and unilateral lower abdominal cramping. During the assessment, she suddenly reports sharp, stabbing pelvic pain that radiates to her right shoulder, accompanied by dizziness and a heart rate of 122 bpm. The nurse should interpret these findings as indicating which condition, and prepare for which priority action?

- A. Inevitable abortion; prepare for immediate administration of misoprostol.
- B. Ruptured ectopic pregnancy; prepare for emergency laparoscopic surgery.
- C. Gestational trophoblastic disease; prepare for immediate suction curettage.
- D. Cervical insufficiency; prepare for emergency cervical cerclage placement.

Q3. A client is being discharged from the outpatient surgical unit following a suction curettage to remove a complete hydatidiform molar pregnancy. Which statement by the client indicates that discharge teaching regarding malignancy prevention has been SUCCESSFUL?

- A. 'I will check my urine for protein daily using a chemical reagent dipstick.'
- B. 'I must strictly avoid getting pregnant for the next year and use a reliable contraceptive.'
- C. 'I will need to return to the clinic next week to have an emergency cervical cerclage placed.'
- D. 'I can begin trying to conceive again as soon as my normal menstrual cycle returns in 3 months.'

Q4. A 16-year-old primigravida at 15 weeks' gestation is admitted with hyperemesis gravidarum. She has been unable to keep any food or fluids down for 3 days, has a 6% loss of body weight, dry cracked lips, poor skin turgor, and urine positive for ketones. What is the PRIORITY initial nursing intervention?

- A. Encourage the client to eat small, frequent meals of dry crackers and toast.
- B. Withhold all oral intake (NPO status) and initiate prescribed intravenous fluid rehydration.
- C. Administer oral antiemetic medications with high-protein supplement drinks.
- D. Request a dietary consultation for a high-calorie, low-fat select menu.

Q5. An Rh-negative, non-sensitized G2P1 client experiences a spontaneous complete abortion at 10 weeks' gestation in the emergency department. Which medication order should the nurse anticipate executing?

- A. Administer a full dose (300 mcg) of Rho(D) immune globulin (RhoGAM) IM in the deltoid within 72 hours.
- B. Administer a mini-dose (50 mcg) of Rho(D) immune globulin (MICRhoGAM) IM in the deltoid within 72 hours.
- C. Administer misoprostol (Cytotec) 800 mcg orally to stimulate further uterine contractions.
- D. Administer betamethasone 12 mg IM to enhance fetal surfactant production.

Q6. A client with severe placental abruption suddenly exhibits bleeding gums, petechiae on her abdomen, and continuous oozing of blood from her intravenous catheter insertion site. Which laboratory results should the nurse anticipate that support a diagnosis of secondary Disseminated Intravascular Coagulation (DIC)?

- A. Increased platelets, increased fibrinogen, decreased PT/aPTT, and negative D-dimer.
- B. Decreased platelets, decreased fibrinogen, prolonged PT/aPTT, and strongly positive D-dimer.
- C. Decreased platelets, increased fibrinogen, prolonged aPTT, and normal D-dimer.
- D. Normal platelets, decreased fibrinogen, shortened PT/aPTT, and positive fibrin degradation products.

12. ANSWER KEY & CLINICAL RATIONALES

Q1: Correct Answer: C. Performing a digital vaginal examination.

Clinical Rationale: The client's clinical presentation (painless, bright-red bleeding in Trimester 3 with a soft, relaxed uterus) is classic for **placenta previa**. A digital vaginal examination is strictly contraindicated because the examiner's finger can physically penetrate the lower uterine segment, disrupt the placenta over the os, and cause rapid, massive, potentially fatal maternal hemorrhage. Ultrasound must always rule out previa first.

Q2: Correct Answer: B. Ruptured ectopic pregnancy; prepare for emergency laparoscopic surgery.

Clinical Rationale: Ectopic pregnancy typically presents with unilateral pelvic pain and spotting 6-8 weeks after a missed period. **Referred shoulder pain** is a critical danger sign indicating **tubal rupture** and active bleeding into the peritoneal cavity, which irritates the diaphragm/phrenic nerve. Accompanied by tachycardia and dizziness, it signals impending hypovolemic shock. This is a surgical emergency requiring immediate laparotomy/salpingectomy.

Q3: Correct Answer: B. 'I must strictly avoid getting pregnant for the next year...'

Clinical Rationale: Molar tissue (GTD) has a high potential to transform into highly invasive, malignant **choriocarcinoma**. Rising serum beta-hCG levels are tracked for 1 year post-evacuation to monitor for choriocarcinoma. A new pregnancy would cause a physiological rise in hCG, making it impossible to determine if rising hCG is due to pregnancy or cancer. Highly reliable contraception is mandatory for 1 year.

Q4: Correct Answer: B. Withhold all oral intake (NPO status) and initiate prescribed IV rehydration.

Clinical Rationale: In severe hyperemesis gravidarum, the priority is to stop all oral intake to allow the irritated gastrointestinal tract and stomach to rest. Forcing oral fluids/foods immediately will trigger further vomiting, worsening dehydration and ketonuria. Oral fluids and dry solids are only reintroduced gradually after the vomiting has completely ceased for 24 hours with IV fluid replacement.

Q5: Correct Answer: B. Administer a mini-dose (50 mcg) of MICRhoGAM within 72 hours.

Clinical Rationale: Combined with early pregnancy losses or abortions occurring up to **12 weeks' gestation**, a **mini-dose (50 mcg/MICRhoGAM)** is strictly indicated for non-sensitized Rh-negative clients. A full dose (300 mcg) is only given for losses occurring after 12 weeks' gestation.

Q6: Correct Answer: B. Decreased platelets, decreased fibrinogen, prolonged PT/aPTT, and strongly positive D-dimer.

Clinical Rationale: DIC is characterized by a rapid depletion of clotting factors (fibrinogen) and platelets due to widespread microvascular clotting, which prolongs PT/aPTT. As the body simultaneously attempts to break down these widespread microclots, large amounts of fibrin degradation products and **D-dimers** are released. A strongly positive D-dimer accompanied by decreased platelets and prolonged clotting times confirms active DIC.

13. QUICK RECALL: CLINICAL MEMORY HOOKS

MATERNAL CONDITION	CLINICAL ONE-LINE MEMORY HOOK & KEY CONCEPT
Spontaneous Abortion	Pregnancy loss before week 20. Count & weigh pads (1 pad/hour = significant). Save passed tissue. RhoGAM within 72 hours if Rh-negative.
Ectopic Pregnancy	Painless/painful spotting 6-8 weeks post-missed period. Referred shoulder pain = Tubal Rupture (Emergency surgical laparotomy). Contraception x 3 cycles.
Molar Pregnancy (GTD)	Non-viable grape-like vesicles. High hCG. Uterus > dates. Preeclampsia before 20 weeks. Contraception strictly for 1 year (tracks choriocarcinoma).
Cervical Insufficiency	Painless dilation in Trimester 2. Pink vaginal discharge. Reinforce internal os with second-trimester cerclage . Monitor for preterm labor.

MATERNAL CONDITION	CLINICAL ONE-LINE MEMORY HOOK & KEY CONCEPT
Placenta Previa	Painless, bright-red bleeding. Soft, relaxed uterus. NEVER perform digital vaginal examinations (hemorrhage risk). Scheduled cesarean birth.
Placental Abruption	Painful, dark red (port-wine) bleeding. Sudden 'knifelike' pain. Rigid, board-like uterus. Left lateral position, 2 large-bore IVs. Watch for DIC.
Disseminated Coagulation (DIC)	Clotting/bleeding imbalance. Triggered by abruption/dead fetus. Labs: ↓ platelets, ↓ fibrinogen, ↑ PT/aPTT, positive D-dimer.
Hyperemesis Gravidarum	Severe vomiting with >5% weight loss. Dehydration, ketosis. Step 1: Strict NPO to rest gut, then IV rehydration and antiemetics.